

RESTORATION MEDICAL

Communications & Marketing Compliance Handbook

Family Medicine | Pain Centre | Medical Aesthetics | Infusion Therapy | Rehab & Recovery

RM-HB-COMMS_v2.1_2026-09-02

Internal document. All staff, contractors, and marketing vendors must read and sign the acknowledgement in Appendix A before publishing any content or communicating on behalf of the clinic.

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Document Control

Version Convention

This handbook uses MAJOR.MINOR versioning:

- **MAJOR (X.0):** structural or policy-substance changes: new regulator obligations, new service lines, changed approval workflow, or any change requiring staff re-acknowledgement. Triggers re-signing of Appendix A by all staff and vendors
- **MINOR (X.Y):** clarifications, contact updates, formatting, corrected references. No re-acknowledgement required; changes announced by internal memo
- File naming: RM-HB-COMMS_vX.Y_YYYY-MM-DD (e.g., RM-HB-COMMS_v1.0_2026-09-02)
- Only the PDF export of the current version is circulated; the Word source is held by the Creative Director. Superseded versions are archived, never deleted
- Scheduled review: every September, or immediately upon any change in CPSO/CNO policy, CASL enforcement guidance, or Google/Meta healthcare ad policy

Revision History

Version	Date	Summary of changes	Approved by
1.0	2026-09-02	Initial release across all five service lines	Creative Director / Medical Director
2.0	2026-09-02	Scope narrowed to communications and marketing compliance only	Creative Director / Medical Director
2.1	2026-09-02	Residual out-of-scope references removed; acknowledgement wording updated for vendor signatories	Creative Director

1. Purpose and Scope

Restoration Medical operates physician-led clinics offering family medicine, chronic pain management, medical aesthetics, IV infusion therapy, intramuscular injections, physiotherapy, and chiropractic care. Because we combine OHIP-insured medicine with uninsured cosmetic and wellness services, our communications sit at the intersection of multiple regulators. A message that is acceptable for a med spa may be a professional misconduct issue for our physicians. This handbook exists so that everyone communicating on behalf of the clinic applies the strictest applicable standard by default.

This handbook applies to:

- All employees, contracted clinicians (physicians, nurses, physiotherapists, chiropractors), and administrative staff
- External vendors: marketing agencies, web developers, social media managers, videographers
- All channels: website, Google Business Profile, Meta (Facebook/Instagram), YouTube, TikTok, email, SMS, print, in-clinic signage and digital signage, phone scripts, and review responses

Core operating rule:

When two rules conflict, follow the stricter one. When in doubt, do not publish. All public-facing content must pass the approval workflow in Section 10 before going live.

2. Regulatory Map: Who Governs What

Every service line and every communication channel has at least one regulator. This table is the master reference.

Authority	What it governs	Applies to
CPSO (College of Physicians and Surgeons of Ontario)	Physician advertising, testimonials, social media conduct, conflicts of interest, delegation, medical directives	All physician content; any ad naming a physician or physician-supervised service
CNO (College of Nurses of Ontario)	Nursing scope, delegation, advertising and professional conduct for RNs/RPNs/NPs	IV therapy, IM injections, aesthetics procedures performed by nurses
College of Physiotherapists of Ontario	PT advertising, title protection, billing transparency	Physiotherapy services and marketing
College of Chiropractors of Ontario (CCO)	Chiropractic advertising standard S-016, title use, claims	Chiropractic services and marketing
Health Canada / Food and Drugs Act	Prohibits advertising prescription drugs to the public beyond name, price, quantity; Schedule A disease claims	GLP-1 program, Botox and other injectables, compounded drugs, IV drug additives
PHIPA (Ontario)	Personal health information privacy, consent for use of patient images and stories	Reviews, testimonials, before/after photos, email lists, surveillance notices
CASL (federal)	Consent, identification, and unsubscribe rules for commercial email and SMS	All email and text marketing, appointment vs. promotional messages

Authority	What it governs	Applies to
Competition Act / Ad Standards (Canadian Code of Advertising Standards)	False or misleading claims, disclaimers, pricing accuracy, influencer disclosure	All advertising, all channels
Google (Ads + Business Profile + YouTube)	Healthcare ad certification, restricted drug terms, personalized ad restrictions, review-gating ban	Search ads, display, YouTube pre-roll, GBP posts and reviews
Meta (Facebook/Instagram)	Health and wellness ad policies, prescription drug restrictions, personal attribute targeting bans, before/after image restrictions	All Meta organic and paid content

Additional jurisdictions to keep on the radar:

- CMTO (College of Massage Therapists of Ontario) if RMT services are added
- HARP Act (X-ray safety) for any fluoroscopy or imaging marketing claims at the Woodbridge pain site
- PIPEDA for non-health personal data collected via website forms and ad pixels
- CRTC telemarketing rules and the National Do Not Call List if outbound calling is ever used
- Law Society advertising rules do not apply, but medico-legal report marketing to lawyers should stay factual

3. Advertising and Marketing: The Core Rules

These rules derive primarily from the CPSO Advertising policy and the General Regulation under the Medicine Act, 1991. Because our clinics are physician-led, they apply to ALL clinic marketing, not just content naming a doctor. The other colleges (CNO, physiotherapy, chiropractic) impose materially similar rules, so one standard covers everyone.

3.1 Universal Do / Don't Table

Permitted	Prohibited
Factual descriptions of services offered, hours, locations, languages spoken (English/Vietnamese)	Testimonials in any advertising: patient quotes, star ratings, or reviews reproduced in ads, on the website, or in social posts
Physician names, specialties, and CPSO-recognized credentials	Superlatives and comparisons: best, top, #1, better than, leading, most advanced
Verifiable, evidence-supported clinical statements	Guarantees or promises of outcomes: results, cure, permanent, pain-free life
Prices for uninsured services, stated accurately and completely	Incentives to seek medical services: discounts, limited-time offers, referral rewards, contests tied to medical treatment
Educational content about conditions and treatment categories	Naming specific prescription drugs or devices in ads (see 3.3 for narrow exceptions)
Compliant before/after photos (see 3.2)	Manipulated, filtered, or cherry-picked imagery presented as typical
'Accepting new patients' announcements	Creating urgency or fear appeals: don't wait, spots filling fast, your health can't wait

3.2 Before and After Photos and Videos

CPSO now permits before/after imagery as an exception to the testimonial ban, but only under all of the following conditions:

- Purpose is accurate, educational information, not hype
- The outcome shown is one a patient can reasonably and typically expect
- No manipulation: identical lighting, angle, distance, expression; no filters, no makeup changes that exaggerate results
- Accompanied by a visible statement that results vary
- The patient gave free and informed written consent, with the right to withdraw, documented in the chart and in the marketing consent log
- Consent obtained separately from treatment consent, never as a condition of treatment or a discount

3.3 Drug and Device Naming

The Food and Drugs Act prohibits advertising prescription drugs to the general public except name, price, and quantity, and prohibits advertising any product to the public as a treatment for Schedule A diseases.

CPSO separately prohibits referencing specific drugs, appliances, or equipment in physician advertising.

Practical rules:

- **GLP-1 / weight management:** never advertise tirzepatide, semaglutide, Mounjaro, Ozempic, Wegovy, or Zepbound by name in public-facing marketing. Market the program: 'physician-supervised medical weight management.' Drug names may be discussed in the exam room and in unbranded patient education provided after intake.
- **Neuromodulators and fillers:** avoid Botox, Dysport, Nuceiva, and filler brand names in ads. Use category terms: 'anti-wrinkle injections,' 'cosmetic injectables,' 'dermal fillers.' Note that Botox is a prescription drug, so the federal ban applies on top of CPSO rules.
- **Devices:** CPSO restricts naming equipment. Describe capability ('fractional CO2 laser resurfacing,' 'RF microneedling') rather than trade names in advertising. Device trade names may appear in neutral, educational service pages where they are generic identifiers of the modality, but keep them out of paid ads and promotional claims.
- **IV therapy:** do not claim IV drips treat, cure, or prevent diseases (immunity boosting claims are a frequent Ad Standards and Competition Bureau target). Describe contents and hydration/wellness support in measured terms; avoid therapeutic claims not supported by evidence.

3.4 Pricing, Discounts, and Promotions

- Insured (OHIP) services: never advertise prices or perks; never offer inducements tied to enrolment as a family practice patient
- Uninsured services (aesthetics, IV, most physio/chiro): prices may be published and must be accurate, all-in, and honoured. Drip pricing violates the Competition Act
- Discounts, packages, and memberships on uninsured cosmetic services are commercially common but sit in tension with the CPSO ban on incentives to seek medical services. House rule: no time-pressure discounts on any service involving a drug, injection, or physician assessment. Package pricing for series treatments (e.g., 6 laser sessions) is acceptable when framed as a treatment plan price, not a promotion
- Never advertise free consultations that are actually insured assessments, and never bill OHIP for visits generated by cosmetic marketing

3.5 Titles and Credentials

- 'Doctor' in patient-facing content is reserved for physicians and chiropractors within their scope; chiropractors must use 'Dr. X, Chiropractor'
- Nurses are identified as RN/RPN/NP per CNO title rules; never 'nurse injector' without the protected title
- 'Medical Director' may be used for Dr. Nguyen; never imply a clinician holds specialist status that CPSO has not recognized
- Only physiotherapists registered with the College may be called physiotherapist/PT

4. Service-Line-Specific Rules

4.1 Family Medicine

- Announcements limited to: accepting new patients, languages, hours, location, enrolment process
- No perks, gifts, or draws for rostering; no advertising that suggests preferential access for paying customers of other service lines
- Keep a hard wall in messaging between insured care and paid services: never imply patients must or should buy uninsured services

4.2 Pain Centre

- No outcome promises (pain-free, guaranteed relief); describe modalities and the assessment process
- Interventional procedures under fluoroscopy: marketing may state the capability but claims about imaging equipment must stay generic (HARP-compliant facility, image-guided injections)
- Opioid-related content requires physician sign-off; never market opioid prescribing
- Referral-based messaging to physicians and lawyers must be factual, without fee-splitting or referral inducements

4.3 Medical Aesthetics

- Follow 3.2 and 3.3 strictly; this line carries the highest regulator and platform scrutiny
- All aesthetic ads must avoid promoting negative body image; Meta and Ad Standards both prohibit content implying a person is inadequate without treatment
- State that treatments follow medical assessment by an authorized prescriber; injectables require a prescription and cannot be sold direct
- Model imagery: use licensed stock or consented models; label simulations as such

4.4 Infusion Therapy and IM Injections

- No disease treatment or prevention claims (immunity, detox, cure hangovers, chelation benefits)
- Describe ingredients factually; vitamin doses may be listed; health claims must be supportable
- Mobile IV marketing must state physician oversight and screening requirements
- Semaglutide/tirzepatide are never added to IV/IM menus in marketing; weight management is marketed only per 3.3

4.5 Physiotherapy and Chiropractic

- Each profession's college prohibits testimonials, superiority claims, and misleading fee advertising, mirroring CPSO
- Chiropractic (CCO Standard S-016): claims must be factual, verifiable, and within scope; no claims of treating non-musculoskeletal conditions (immune function, colic, etc.)
- Physiotherapy: advertise services, not guaranteed recovery timelines; direct-billing statements must be accurate about which insurers are supported

5. Platform Rules: Google, Meta, YouTube

Platforms enforce their own healthcare policies on top of Canadian law. Account suspensions are common in aesthetics and weight-loss; violations can kill an ad account permanently.

5.1 Google Ads and Google Business Profile

- Healthcare-related advertising is restricted; certain categories require certification. Prescription drug terms in ad text, keywords, or landing pages will be disapproved for Canadian targeting
- No remarketing/personalized audiences based on health states (pain sufferers, weight loss seekers). Use contextual and geographic targeting instead
- Landing pages count: a compliant ad pointing to a page that names prescription drugs will still be disapproved
- Google Business Profile: never offer anything in exchange for reviews, never selectively solicit happy patients (review gating), respond to all reviews per Section 7
- Local Services and Maps content must match the licensed legal entity and real address

5.2 Meta (Facebook and Instagram)

- Prescription drugs cannot be promoted; weight-loss ads must not show idealized before/after body imagery or promote negative self-perception
- Cosmetic procedure ads must target 18+; set this at the ad set level for all aesthetics campaigns
- No zoomed-in 'problem area' imagery (skin flaws, fat, cellulite close-ups)
- Personal attribute rule: ads may not imply knowledge of a person's condition ('Struggling with chronic pain?' fails; 'Our Pain Centre offers image-guided treatment' passes)
- Health and wellness data restrictions: do not send patient or appointment data to the Meta Pixel/CAPI from booking pages; strip pixels from any authenticated or intake flow

5.3 YouTube

- YouTube follows Google Ads healthcare policy for paid placement; organic clinical content must avoid medical misinformation categories
- Procedure videos may be age-restricted; upload with accurate titles, no clickbait outcome claims
- Patient-featured video requires the full PHIPA consent stack (Section 8) plus a signed media release
- Comments on clinical videos: disable or actively moderate; never let comment sections become testimonial walls that we curate (deleting only negative comments converts them into advertising we control)

5.4 TikTok and Emerging Platforms

- Apply the strictest existing standard by default; TikTok bans weight-loss drug promotion and heavily restricts cosmetic procedure ads to minors
- Trends and audio memes must never involve patient areas, PHI, or staff in clinical contexts without approval

6. CASL: Email and SMS Rules

CASL applies to every commercial electronic message (CEM): any email or text that encourages participation in a commercial activity, including promotions for aesthetics, IV therapy, or paid programs. Penalties reach \$10 million per violation for organizations.

6.1 What Is and Is Not a CEM

Message type	CASL status	Rules
Appointment reminders, recall notices, test result notifications	Not a CEM (transactional/care)	No consent needed beyond PHIPA; do NOT add promotional content or they become CEMs
Newsletter with service promotions, new treatment announcements, price lists	CEM	Full CASL compliance: consent + identification + unsubscribe
Mixed message (reminder + 'ask us about our IV menu')	CEM	The promotional line contaminates the whole message. Keep streams separate

6.2 The Three Requirements for Every CEM

- **1. Consent.** Express consent (checked box, never pre-checked) is gold: it never expires. Implied consent from an existing business relationship expires 2 years after the last transaction. Track the consent type, date, and source for every address. An inquiry gives only 6 months of implied consent
- **2. Identification.** Legal clinic name, mailing address, and a contact method (phone/email/web) in every CEM
- **3. Unsubscribe.** A working, no-cost, no-login unsubscribe in every CEM, honoured within 10 business days (our standard: 48 hours). Applies to SMS too: reply STOP must work

6.3 House Rules

- Never buy, rent, or scrape email lists
- Patients enrolling for medical care are NOT automatically marketing subscribers; the marketing opt-in is a separate, unchecked box on intake forms (EN and VI versions)
- Segment: clinical messages from contact@restorationmedical.ca; marketing from a dedicated address so unsubscribes never suppress clinical communication
- Vietnamese-language CEMs carry the same footer and unsubscribe requirements
- Referral programs: 'forward to a friend' incentives make the sender liable under CASL; do not run them by email

7. Reviews, Testimonials, and Reputation

7.1 The Line

- Patients posting freely on Google, RateMDs, or social media is legal and fine
- The clinic reproducing, quoting, boosting, or embedding those reviews in its own website, ads, or posts converts them into prohibited testimonials
- Curating comments (deleting negatives, keeping positives) makes third-party content our advertising; only discriminatory or abusive comments may be removed

7.2 Responding Under PHIPA

Never confirm someone is a patient, even when they identify themselves. Every response uses the neutral pattern:

English closing: 'With care, / The Restoration Medical Team' | Vietnamese: 'Tran trong, / Doi ngu Restoration Medical'

- Thank the reviewer without referencing any visit, treatment, or detail
- Invite offline resolution through contact@restorationmedical.ca or the clinic line
- Negative reviews get one professional response, never a rebuttal thread; escalate threats or clinical complaints to the Medical Director same day
- Never ask staff, family, or vendors to post reviews; never trade anything of value for a review (Google policy + Competition Act deceptive marketing provisions on fake/incentivized reviews)

8. Privacy (PHIPA) in Marketing and Communications

- Patient PHI is never used for marketing without express, documented, revocable consent obtained separately from care
- Photography/video in clinic: no patient is ever captured without signed media consent; waiting-room b-roll requires scheduling a closed set. Surveillance notice posters remain posted per our PHIPA surveillance notice
- Email lists for marketing are consent-based (Section 6) and stored in the marketing platform, not exported from OSCAR Pro. OSCAR/Ocean data is used for care communications only, except where a patient has expressly opted into marketing
- Staff phones: no patient info, schedules, or charts photographed or messaged on personal devices; no patient discussion in personal social accounts
- Breach protocol: suspected privacy breach in any marketing context (wrong email merge, visible CC list, patient visible in a post) is reported to the Privacy Officer immediately; IPC notification assessed within 24 hours; mass emails always use BCC or a proper ESP
- AI tools: no PHI is ever entered into consumer AI tools; marketing copy generation must use de-identified scenarios only

9. Social Media Conduct (All Staff)

- Only designated staff post to clinic accounts; credentials live in the password manager, never in personal browsers
- Clinicians engaging professionally online remain bound by their college's social media guidance at all times; the CPSO treats promotional posts by or on behalf of physicians as advertising
- Personal accounts: staff may not identify patients, share clinic interior photos containing PHI, discuss cases (even de-identified 'story time' content), or present personal opinions as clinic positions
- No medical advice in comments or DMs; standard reply directs to booking or the clinic phone
- Influencer or creator collaborations require a written agreement, Ad Standards disclosure (#ad), no testimonial framing, and pre-approval of every deliverable; gifted treatments to influencers are treated as incentivized endorsements and are not permitted for physician-supervised services
- Crisis rule: if a post attracts regulator attention, media inquiry, or viral criticism, freeze the account and escalate to the Medical Director and Creative Director; no replies until alignment

10. Content Approval Workflow

Content tier	Examples	Required sign-off
Tier 1: Clinical claims	Any content naming treatments, drugs categories, outcomes, before/after imagery, weight management program	Medical Director + Creative Director
Tier 2: Promotional	Pricing pages, campaigns, landing pages, CEMs, paid ads	Creative Director + compliance checklist
Tier 3: Operational	Hours changes, closures, hiring posts, community events	Clinic Manager

10.1 Pre-Publication Checklist (attach to every Tier 1-2 approval)

- No testimonials, superlatives, guarantees, or urgency devices
- No prescription drug or restricted device names in ad or landing page
- Before/after imagery meets all 3.2 conditions with consent on file
- Prices accurate and all-in; disclaimers legible on mobile
- CASL footer and unsubscribe present (if CEM); consent segment verified
- Platform policy check for the specific channel (Section 5)
- EN and VI versions carry identical compliance elements
- Titles and credentials correct for every named clinician

11. Complaints, Incidents, and Regulator Contact

- Any letter, call, or email from CPSO, CNO, another college, the IPC, the Competition Bureau, Ad Standards, or the CRTC goes to the Medical Director and Creative Director the same day; no one else responds
- Ad account suspensions (Google/Meta): stop all campaigns, document the flagged asset, remediate before appeal; never create workaround accounts (permanent ban risk)
- Marketing-related patient complaints are logged like clinical complaints and reviewed monthly
- Annual compliance review each September: this handbook, consent logs, CASL records, and ad account policy status

Appendix A: Acknowledgement

I confirm that I have read and understood the Restoration Medical Communications & Marketing Compliance Handbook (v2.1, per the Document Control section). I agree to follow its requirements and to route all public-facing content through the approval workflow in Section 10. I understand that violations may result in removal of publishing access and termination of the engagement.

Name: _____ Role: _____

Signature: _____ Date: _____

Appendix B: Quick Reference Card

Situation	Rule in one line
Patient leaves a glowing Google review	Reply with the neutral PHIPA-safe template; never repost it anywhere
Vendor drafts an ad saying 'Best pain clinic in Vaughan'	Reject: superlative + comparison, prohibited
Instagram post about the GLP-1 program	Program language only; no drug names, no before/after bodies, 18+ targeting
Email blast for new IV menu	CEM: consent segment only, ID block + unsubscribe, no disease claims
Videographer wants waiting-room b-roll	Closed set, signed media consents, surveillance notices posted
Patient asks staff to be in a TikTok	Written media + marketing consent first; Medical Director approval
Someone offers to sell a patient email list	Never. CASL + PHIPA violation
Regulator letter arrives	Medical Director + Creative Director same day; nobody else replies